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**YOU DON'T NEED
CYSTS TO HAVE
PCOS.
*or insulin resistance***

*What else drives it, and
what nobody tested*

THE WELLNESS WAY · MASON

BEFORE YOU START

One thing worth watching for.

Most of what follows is about why you feel how you feel. This page is the exception. It is the one part of PCOS that is genuinely time-sensitive.

Periods that have stopped, or come only a few times a year

When the uterine lining is built up by estrogen month after month and never shed, that tissue keeps accumulating. Over years this raises the risk of changes in the lining, and it is the reason absent periods are worth a physician visit rather than a wait-and-see.

Bleeding that is unusually heavy, or lasts beyond a week

Especially if it is new for you, or if you are soaking through tampons hourly.

Bleeding between periods, or after intercourse

Worth getting looked at rather than explained away.

A note on what this is

This guide is education, written and reviewed by Dr. Ryan DeNome, a chiropractic physician. It is not a diagnosis and it does not replace care from your physician. Where something needs a medical evaluation, we say so.



Before we start.

You have probably been handed one explanation. This guide is about the rest of them.

Most women get told PCOS is a blood sugar problem, handed a prescription, and sent home. For some of you that is genuinely part of the story.

But somewhere between a third and a half of women with PCOS do not have insulin resistance at all. Plenty never develop cysts. The name describes a cluster of findings, not a single disease, and two women can arrive at the same diagnosis by very different routes. That matters, because if the explanation you were given is not your explanation, the plan built on it was never going to work.

What follows is what else we look at, and why. Some of it is well established. Some of it is promising and unproven, and I have tried to be clear about which is which.

DR. RYAN DENOME, D.C.



Most of these get explained away alone.

Individually each has an easy answer. Together they describe something.

- 1 **A sex drive that never cycles**
Hormones should rise and fall across the month. Constant, or absent, both mean the cycle has flattened.
- 2 **Tired in a way sleep does not fix**
Sleep problems are common in PCOS, and the relationship runs both directions.
- 3 **Hair where you did not ask for it**
Chin, lip, abdomen, inner thighs. Usually androgens, though not every woman has them.
- 4 **A period you cannot predict**
If the brain does not know when to release an egg, it does not know when to shed the lining.
- 5 **Heavy and painful when it arrives**
Lining that has been building for months has more to shed.
- 6 **Weight that settles at the waist**
Studies find high visceral fat regardless of the scale, so it shows up in slim women too.
- 7 **Deep, painful acne along the jaw**
Usually the week before a period. Large, slow to heal, deep rather than on the surface.
- 8 **Cysts, or difficulty conceiving**
Seventy to eighty percent have trouble conceiving. Years of not being believed is part of that.
- 9 **Anxious, low, or just not yourself**
Mood changes travel with PCOS reliably. Broken sleep and heavy cycles are reason enough on their own.

Not every woman with PCOS has cysts.

Not every woman with PCOS has insulin resistance.



It is not called PCOS any more.

In May 2026 an international consensus of fifty-six professional and patient organisations formally renamed it in The Lancet. Polycystic ovary syndrome became polyendocrine metabolic ovarian syndrome. PMOS.

The reason was accuracy. What get called cysts are not cysts, they are ordinary follicles. And you can have the condition without them, or have them without the condition. The old name pointed at the one feature that is neither necessary nor sufficient to diagnose it.

- ① **Polyendocrine**
More than one hormone system is involved. Not one gland having a bad day.
- ② **Metabolic**
Type 2 diabetes risk runs roughly four times higher, and that holds when women are matched for body weight. It is not incidental to the condition.
- ③ **Ovarian**
Kept deliberately. The consensus rejected framing this as purely metabolic, because it is still a reproductive condition too.

Both names are currently correct

You will see both names for a while. The transition runs to 2028 and PCOS is not yet an error, so we use it in this guide where it aids recognition. The change was also not unanimous, and the dissent argued the new name still leans too far toward the ovary.



WHERE INSULIN ACTUALLY FITS

Real for many.

Not the whole story.

Insulin resistance is a genuine part of PCOS and worth taking seriously. Insulin tells your body what to do with the sugar in your blood, and when cells stop responding, the knock-on effects reach androgens, weight and cycles.

50 to 70%

of women with PCOS have insulin resistance, by the figure The Wellness Way cites

30 to 50%

do not have it, and for them a plan built only on blood sugar is aimed at the wrong thing

Either way

you cannot tell which group you are in without measuring it

This is why the first question is not what to cut. It is whether insulin resistance is present at all, because the answer changes what the rest of the plan should be.

If sugar is the problem, we will find it and address it. If it is not, we keep looking rather than blame you for a diet that was never going to move your numbers.



Inflammation is a finding, not an answer.

This is one of the better-supported findings in PCOS. Two large reviews, covering more than ninety studies, found C-reactive protein consistently higher in women with PCOS. When researchers looked only at women who were not overweight, the difference got larger rather than smaller. So it is not simply a consequence of body weight.

But a raised CRP does not tell you what is raising it. It is closer to a smoke alarm than a diagnosis. The useful question, and the one this guide is built around, is what is producing it.

- 1 **What your gut is dealing with**
Bacterial balance, and whether estrogen your liver already cleared is being sent back into circulation.
- 2 **Foods you react to without noticing**
A reaction does not have to be dramatic to be constant, and most produce no symptom you would connect to it.
- 3 **Thyroid function and thyroid antibodies**
An immune system attacking the thyroid is an inflammatory process, and a TSH alone will not show it.
- 4 **What you are exposed to**
Compounds in plastics, personal care products and the food supply that act on hormone receptors.



THE PART ALMOST NOBODY CHECKS

Your gut can undo the work your liver did.

Your liver deals with used estrogen by tagging it, which switches it off and marks it for disposal through the bile and out. That is the exit route.

Certain gut bacteria make an enzyme that snips that tag back off. The estrogen becomes active again and is reabsorbed instead of leaving. Researchers have taken thirty-five of these bacterial enzymes, tested them directly, solved their structures, and blocked the reaction with an inhibitor. The mechanism is not a theory. This enzyme is called Beta-glucuronidase and if you have not taken a stool test to know if this is happening that is a great place to start.

We look at it because it is measurable and because it is upstream of the hormone panel everyone stops at.



WHAT THE GUT STUDIES SHOW

The differences are real. The direction is not settled.

Twenty-eight studies covering nearly two thousand women found measurably lower diversity in the gut bacteria of women with PCOS. The finding is consistent, and the size of the difference is modest rather than dramatic.

WHAT IS ESTABLISHED

- Diversity is reduced in PCOS across many studies
- Specific groups of bacteria recur across cohorts
- In animals, transferring the bacteria transfers part of the phenotype

One large study found that the higher a woman's androgens, the lower her gut diversity, and concluded the hormones may be driving the bacteria rather than the other way round. That possibility is at least as well supported as the reverse.

We test the gut because it is one of the few things on this list you can actually change, not because we can promise you which way the arrow points.



Some things bind the receptor from outside.

Your hormones work by fitting a receptor, like a key in a lock. A number of manufactured and naturally occurring compounds fit those same locks well enough to turn them, which is why they are called endocrine disruptors.

- ① **BPA and plastics**
Of the environmental compounds studied in PCOS specifically, bisphenol A has the most support. A systematic review of pollutants and PCOS found a positive association in the majority of studies that looked.
- ② **Phthalates**
Found in fragrances, personal care products and soft plastics. They interfere with hormone signalling, and unlike most exposures on this list they are straightforward to reduce.
- ③ **Zearalenone, from toxic mold exposure**
A compound produced by molds that grow on corn and grain. It binds both human estrogen receptors, which is well documented.

The pattern here is worth noticing. Nobody asks what you are exposed to before telling you your hormones are imbalanced, and exposure is one of the few parts of this you can act on this week.



THE REACTIONS YOU CANNOT FEEL

A reaction does not have to be dramatic to matter.

Most people picture hives and an epi-pen. The far more common version is a food you eat several times a week that provokes a low-level immune response every time, and that you have never once connected to how you feel.

The Wellness Way's own position on this is blunt. Very few food allergies produce noticeable symptoms, which means you will not identify them by paying closer attention. Clinics see reactions to foods nobody would suspect, including lettuce, rice and black pepper.

Why guessing does not work

This is why elimination diets built on guesswork disappoint. They remove foods you never needed to remove, leave in the ones costing you something, and then get blamed on willpower when nothing changes.

Food reactions are inflammatory, and inflammation is measurable in PCOS. That is the whole reason this page is in a hormone guide.



THE GLAND THAT TRAVELS WITH IT

A TSH on its own is not a thyroid panel.



Autoimmune thyroid disease turns up more often in women with PCOS than chance would explain, and the association runs both directions. It is a co-occurrence rather than a proven cause, and part of it tracks with weight and thyroid function. But it is consistent enough that it changes what we order.

What a TSH does not tell you

TSH is a message from the pituitary telling the thyroid what to do. It is not a measure of what your thyroid is making, whether you convert it to the active form, or whether your immune system is attacking it. A complete picture includes free and total T3 and T4, reverse T3, and thyroid antibodies.

This is the single most common thing we find that a previous workup missed, and it is missed because only one number was run.



BRING THIS WITH YOU

Three things worth writing down.

Ten minutes on this before any appointment, ours or anyone else's, is the difference between fifteen minutes of recall and a conversation that goes somewhere.

1 Your cycle, as it actually is

Not what it should be. How many periods in the last twelve months, how long, how heavy, how predictable.

2 What changed, and roughly when

Weight, skin, hair, mood, energy. Put a rough date on each. Patterns show up in the timeline that do not show up in a symptom list.

3 Everything you are taking

Medications, supplements, and birth control, with how long you have been on each.

Bring your prior labs. If it was run in the last year, we would rather read it than repeat it.



ASK THESE OF US, OR OF ANYONE

Ask harder questions.

If a practitioner cannot answer these plainly, that tells you something useful.

“ *Do I actually have insulin resistance, or is that being assumed?*

“ *What are we running besides a TSH and a hormone panel?*

“ *Are we checking inflammation, and are we checking the gut?*

“ *If my labs come back normal again, what is the next step rather than the end of the conversation?*

What does this cost, and what will you do differently depending on the result?

That last one matters most. A test that will not change what happens next is a test worth skipping.

Your next step is a free 15 minute consultation where you will talk to Dr. Ryan and we will decide if you are a good fit to work with us! If you just want to dive right in then you are more than welcome to schedule a new patient visit

thewellnesswaymason.com · (283) 223-8376 · New patient visit \$99 flat

